

HYPERTENSION: ROUTINE CARE

Assess the patient with hypertension

Assess	When to assess	Note
Symptoms	Every visit	Manage symptoms as on symptom pages. Ask about symptoms of heart failure ↗ 135, ischaemic heart disease ↗ 137 or stroke/TIA ↗ 136.
Pregnancy	Women of child bearing age: every visit	- If pregnancy diagnosed, stop ACE-inhibitors (like enalapril), give instead methyldopa 250mg 8 hourly and refer to high-risk antenatal clinic. - If planning pregnancy, refer to doctor. - Assess patient's contraceptive needs ↗ 154.
Alcohol/drug use	At diagnosis	In the past year, has patient: 1) drunk ≥ 4 drinks ¹ /session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any ↗ 142.
Adherence	Every visit	If BP is not controlled, assess and support adherence ↗ 173.
BP control	- Every visit (check 2 readings) - For correct method ↗ 132.	- If BP < 140/90, BP is controlled: continue current treatment and review 6 monthly. - If BP ≥ 140/90, BP is not controlled: if adherent, step up treatment ↗ 134. If not adherent, give support ↗ 173 and review in 1 month. - If ≥ 180/110: also check if needs urgent attention ↗ 132. - If SBP consistently ≤ 110, consider decreasing dose or medications.
Weight, BMI, waist circumference	- Weight: at every visit - BMI, waist circumference: at diagnosis	- BMI = weight (kg) ÷ height (m) ÷ height (m). - Aim for BMI < 25 and waist circumference < 80cm (woman) or < 94cm (man).
CVD risk	At diagnosis, then depending on risk	Assess CVD risk ↗ 127.
Urine dipstick	At diagnosis, then yearly	If 1+ proteinuria on dipstick, check creatinine and eGFR. If glucose on dipstick, screen for diabetes ↗ 17.
Diabetes risk	Yearly and if glucose on urine dipstick	If known diabetes ↗ 130. If not known with diabetes, check glucose ↗ 17.
Creatinine (eGFR)	- If 1+ proteinuria on dipstick - Yearly if: CVD², hypertension for ≥ 10 years, uncontrolled hypertension, eGFR < 60	- If eGFR < 30, refer. - If eGFR < 60, send urine to lab for albumin/creatinine ratio and refer to doctor: if ratio > 3, discuss/refer. - If creatinine increases by > 20%, stop enalapril and refer to doctor.
Potassium	- If on enalapril or eGFR < 30: at diagnosis - If on spironolactone or eGFR < 30: 6 monthly	If potassium > 5.0, stop enalapril and spironolactone and refer to doctor.

Advise the patient with hypertension

- Educate the patient that blood pressure changes slightly during the day and night: hypertension is when it stays high, above a certain level. S/he may not have any symptoms.
- Emphasise salt restriction ≤ 1 teaspoon/day, regular physical exercise (150 minutes/week), weight reduction and smoking cessation. If patient smokes, encourage to stop ↗ 141.
- Advise to avoid NSAIDs (e.g. ibuprofen) and combined oral contraceptive.
- Explain importance of adherence and that patient will need lifelong hypertension care to prevent stroke, heart disease, eye disease and kidney disease.
- If newly diagnosed, refer for community health worker support.
- Advise patient on hydrochlorothiazide to limit exposure to sunlight and use sunscreen when exposed to sunlight.

Health for All

↗ 82

¹One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer. ²Cardiovascular disease (CVD) includes ischaemic heart disease, peripheral vascular disease and stroke/TIA.